

CENTRAL CALIFORNIA
EMERGENCY MEDICAL SERVICES
A Division of the Fresno County Department of Public Health

Manual	Emergency Medical Services Administrative Policies and Procedures	Policy Number 530.44
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VENTILATORS (AUTOMATIC TRANSPORT) - ATV		
References	California Code of Regulations Title 22, Division 9, Chapter 3.3	Effective 06/01/2025

I. PURPOSE

To authorize paramedics to use Automatic Transport Ventilators (ATV) during interfacility transports (IFT).

II. POLICY/

- A. Use of transport ventilators by authorized Paramedics is limited to the following categories of patients:
 1. Ventilator dependent adult (greater than 14 years of age) patients from a non-Intensive Care Unit to another healthcare facility.
 2. Ventilator dependent adults (greater than 14 years of age) being transported from an acute care setting to home, skilled nursing facility, or rehabilitation facility.
 3. Ventilator dependent adult (greater than 14 years of age) patients from a home health, long-term care facility, skilled nursing facility, or rehabilitation center to an acute care facility.
- B. Transportation of patients using a transport ventilator shall only occur for patients already receiving mechanical ventilation with stable settings (to the extent possible) and parameters already established by a physician. Paramedics shall not initiate ventilator support.
- C. Paramedics utilizing ATVs must successfully complete an approved training course for competency for use of an automatic transport ventilator. Paramedics must complete a refresher course on the use of a transport ventilator at least every two years. Documentation of such training should be maintained by the ALS provider agency.
- D. Ventilator support must be regulated by an ATV familiar to the Paramedic.
- E. Provider agencies using transport ventilators must follow manufacturer recommendations for maintenance, cleaning, testing, and updating the transport ventilators.

III. PROCEDURE

- A. Written transfer orders for ventilator settings must be established by a physician and continued during transport. These orders must provide for maintaining and adjusting ATV settings during transport and must be attached to the ePCR.
- B. The ATV used must be able to match the existing ventilator settings and shall include the following features:
 1. Modes: Assist Control (AC) and Synchronized Intermittent Mandatory Ventilation (SIMV)
 2. Ventilation rate control

Approved By	Daniel J. Lynch	Revision
EMS Director	(Signature on File at EMS Agency)	01/09/2026
EMS Medical Director	Miranda Lewis, MD (Signature on File at EMS Agency)	

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3. Tidal volume control
 4. FiO₂
 5. Positive End-Expiratory Pressure (PEEP) Control
 6. Inspiratory time control
 7. Peak airway pressure gauge
 8. Peak airway pressure alarm and disconnect alarm
- C. Turn on the transport ventilator and enter the ventilator settings prior to transferring the patient onto the ATV.
 - D. Observe the patient on the transport ventilator for approximately 5 minutes to ensure that the patient is tolerating the transport ventilator prior to leaving the facility.
 - E. Continuous waveform capnography, continuous pulse oximetry, and cardiac monitoring shall be maintained throughout transport. Values and vital signs shall be documented every 10 minutes and any time there is a change in patient condition or adjustment of ventilator settings.
 - F. Check for appropriate placement of the airway device initially and every time the patient is transferred, anytime there is a change in condition of the patient, or anytime there is a risk of dislodgement.
 - G. Use inline suctioning with sterile technique as needed to clear the airway of secretions.
 - H. Observe for potential complications of mechanical ventilations.
 1. Hypoxia: DOPE mnemonic (Displacement of tube, Obstruction of tube, Pneumothorax, Equipment failure).
 2. High pressures: obstruction (secretions, kinked tube, biting tube), air stacking due to vent settings, pneumothorax, bronchospasm, coughing.
 3. Low pressures: low flow, leak in the circuit, leaking endotracheal tube cuff.
 4. Patient not tolerating ventilator: coughing, gagging, and bucking the ventilator.
 - I. One-time dose of midazolam 4 mg IV/IO/IM may be administered for agitation in ventilated patients with systolic blood pressure >90 mm Hg. Contact base for additional medications.
 - J. Inline nebulized albuterol 2.5 mg/3 ml may be administered as needed for wheezing due to bronchospasm. May repeat twice.
 - K. If unable to resolve ventilator issues and/or the patient is deteriorating due to ventilator complications, disconnect the ventilator and initiate ventilations using a bag-valve mask.
 - L. If an endotracheal tube or tracheostomy tube becomes dislodged, initiate bag-valve ventilations.
 - M. Document ventilator settings and patient condition on arrival to receiving facility.
 - N. A quality improvement report must be submitted within 24 hours in the event of any ventilator complication including patient deterioration, endotracheal tube dislodgement, or need for manual ventilations during transport.